RTT within 18 weeks

Forecast for September 2026 · data through August 2026

The latest RTT within 18 weeks result was 65.2%. For the next release, it is expected to remain broadly stable at 65.1%. Current-level seasonal and underlying-trend effects are small or offsetting, leaving the outlook broadly stable. The current projection is off course for 70% by March 2027. Across providers, sustained favourable signals outnumber adverse ones (79 to 10).

LATEST · AUGUST 2026	FORECAST · SEPTEMBER 2026	ACTUAL
65.2%	65.1%	—
0.5pp above forecast	80% expected range 63.9%–66.3%	Not yet released
		Off course · 70% by March 2027

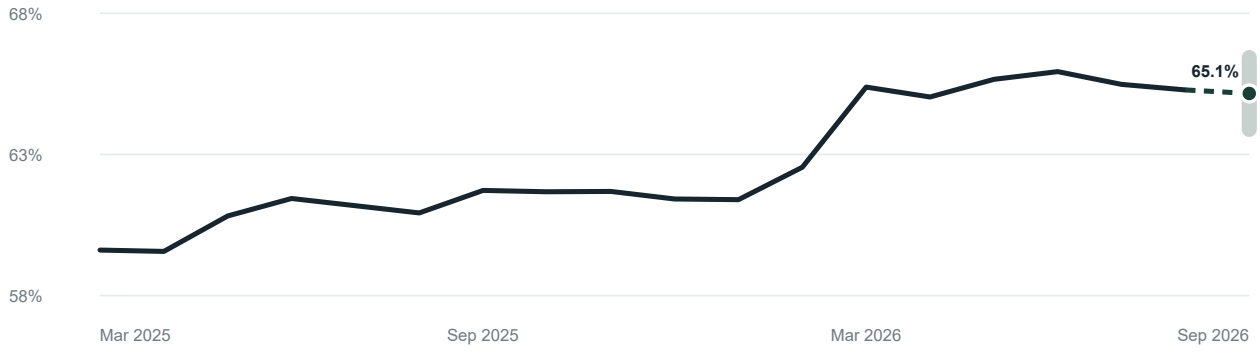
WHAT CHANGED?

New RTT periods exceeded admitted and non-admitted completions by 279,135. This is consistent with inflow exceeding recorded outflow; the residual also includes transfers, corrections and reporting differences.

Waiting list	Over 18 weeks	New periods less completions
+26,521	+15,663	+279,135

Recent performance and next release

The vertical band is the 80% expected range.



Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.



Benchmark: 92% constitutional standard. 3.8pp below milestone

Data: NHS England referral to treatment waiting times. **Targets:** Medium Term Planning Framework national target · Medium Term Planning Framework final target

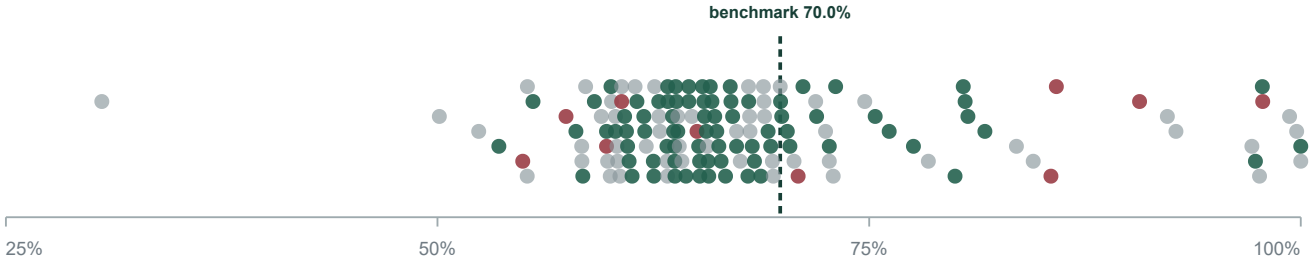
Method: Backtest-weighted ensemble of current-level seasonal movement, damped annual drift and a damped recent seasonal trend; at least 2 components are required. The final fit uses 229 consecutive months. Accuracy is tested on 36 rolling one-month-ahead forecasts. The 80% range uses empirical historical error quantiles from 24 forecast errors. Weights use inverse rolling RMSE and are shrunk toward equal weights. The pre-release commentary compares the current-level seasonal component with the combined forecast. Provider distribution and trajectory watch are shown on page 2.

RTT within 18 weeks

Distribution and trajectory watch · August 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



Trajectory watch

Six actual months are compared with one forecast path fixed before that window. Both lines begin at the same observed value. A low performer can be improving, while a high performer can be deteriorating. Status: Actual performance is compared with the monthly path forecast six months earlier. The displayed figure is the average gap across those six months.

139	79	10	50
providers assessed	sustained favourable	sustained adverse	without an active signal

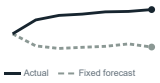
Largest sustained favourable moves

OXLEAS NHS FOUNDATION TRUST
Latest 100.0% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



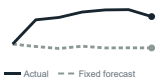
29.3pp
BETTER THAN FIXED PATH

COUNTESS OF CHESTER HOSPITAL NHS FOUNDATION TRUST
Latest 68.4% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



13.3pp
BETTER THAN FIXED PATH

LIVERPOOL WOMEN'S NHS FOUNDATION TRUST
Latest 61.3% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



12.5pp
BETTER THAN FIXED PATH

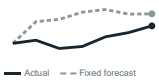
Largest sustained adverse moves

BRADFORD DISTRICT CARE NHS FOUNDATION TRUST
Latest 55.2% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



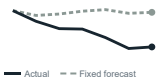
11.1pp
WORSE THAN FIXED PATH

SUSSEX COMMUNITY NHS FOUNDATION TRUST
Latest 60.9% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



8.7pp
WORSE THAN FIXED PATH

HAMPSHIRE AND ISLE OF WIGHT HEALTHCARE NHS FOUNDATION TRUST
Latest 85.6% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window



4.4pp
WORSE THAN FIXED PATH

Trajectory-watch rule. Six actual months are compared with the corresponding path fixed before the six-month window. A signal requires a material average gap, at least five of six months in the same direction, and the latest month confirming that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.